National Cancer Control Programme (NCCP) 00815 · clinician draft

Weekly paclitaxel + trastuzumab → trastuzumab continuation

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

First 12 weeks · paclitaxel + trastuzumab

Active medicines: Paclitaxel + Trastuzumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (National Cancer Control Programme (NCCP) suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- Trastuzumab formulation and observation instructions remain separate from taxane premedication.
- Growth-factor support during weekly paclitaxel is individualised; do not automatically carry the mandatory dose-dense AC prescription into this phase.
- Confirm IV versus locally approved subcutaneous (SC) trastuzumab. SC is fixed 600 mg every 21 days without a loading dose; IV and SC missed-dose instructions are different.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00815 v3 · p7 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.18 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Adjuvant continuation · trastuzumab alone

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine chemotherapy antiemetics or paclitaxel premedicines carry into trastuzumab-only continuation. Review previous symptoms/reactions individually.
- Continue the cardiac-monitoring clock and confirm the prescribed route, interval and missed-dose rules.
- Confirm intravenous (IV) versus locally approved subcutaneous (SC) trastuzumab. SC is fixed 600 mg every 21 days without a loading dose; IV and SC missed-dose instructions are different.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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